



OFFICE OF THE POLICE COMMISSIONER

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August 6, 2009

Memorandum for: Deputy Commissioner, Trials

Re: **Police Officer Rafael Montanez**
Tax Registry No. 918022
107 Precinct
Disciplinary Case Nos. 83572/08, 84579/08 & 84986/09

The above named member of the service appeared before Assistant Deputy Commissioner John Grappone on June 26, 2009 and was charged with the following:

DISCIPLINARY CASE NO. 83572/08

1. "This case was calendared and scheduled for trial. At outset of this trial, however, the Assistant Department Advocate informed this Court that the charges and specifications were never served on the Respondent. Consequently, the Department conceded that since the charges were not timely served this case is time barred from prosecution by the Statute of Limitations. This Court, therefore, deems this case **DISMISSED**."

DISCIPLINARY CASE NO. 84579/08

1. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on or about eight (8) occasions between about July 28, 2008 and about September 5, 2008, while designated as "Chronic Sick – Category B," and while assigned on restricted duty status to perform a tour of duty on each occasion from 0700 hours through 1523 hours, did fail and neglect to perform said assigned tours, after having been found fit by Department Medical Division physicians and directed, on or about the day preceding each such occasion, to report to work, in that said Police Officer, on each such occasion, reported sick during said assigned tour or within a few hours of the commencement of the following assigned tour, claiming sickness due to the existing condition reviewed and evaluated the previous day by said Department Medical Division physicians. (As amended at trial)

P.G. 203-05, Page 1, Paragraph 1

**PERFORMANCE ON DUTY –
GENERAL REGULATIONS**

2. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on July 28, 2008, having been found fit for restricted duty and ordered by Department Orthopedist Arnold Goldman, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

P.G. 203-03, Page 1, Paragraph 2

**COMPLIANCE WITH ORDERS -
GENERAL REGULATIONS**

P.O. RAFAEL MONTANEZ

DISCIPLINARY CASE NOS. 83572/08, 84579/08, 84986/09

3. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on August 6, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

P.G. 203-03, Page 1, Paragraph 2

**COMPLIANCE WITH ORDERS –
GENERAL REGULATIONS**

4. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on August 15, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

P.G. 203-03, Page 1, Paragraph 2

**COMPLIANCE WITH ORDERS –
GENERAL REGULATIONS**

5. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on August 29, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

P.G. 203-03, Page 1, Paragraph 2

**COMPLIANCE WITH ORDERS –
GENERAL REGULATIONS**

6. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on September 2, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

P.G. 203-03, Page 1, Paragraph 2

**COMPLIANCE WITH ORDERS –
GENERAL REGULATIONS**

DISCIPLINARY CASE NO. 84986/09

1. Said Police Officer Rafael Montanez, while assigned to the 107 Precinct, on or about August 14, 2008, wrongfully did secure his 107 Precinct Department locker with a combination lock displaying the serial number of said lock.

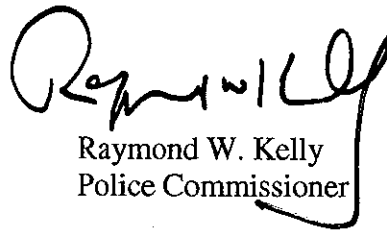
P.G. 203-03, Page 1, Paragraph 2

**COMPLIANCE WITH ORDERS –
GENERAL REGULATIONS**

In a Memorandum dated January 29, 2007, Assistant Deputy Commissioner Grappone DISMISSED Disciplinary Case No. 83572/08, found the Respondent GUILTY of all Specifications in Case No. 84579/08, and GUILTY of the sole Specification in Case No. 84986/09. Having read the Memorandum and analyzed the facts of these matters, I approve the findings, but disapprove the recommended penalty.

P.O. RAFAEL MONTANEZ
DISCIPLINARY CASE NOS. 83572/08, 84579/08, 84986/09

The Respondent's acts of misconduct, combined with his poor discipline and performance record, merits a greater penalty than was recommended. Therefore, the Respondent is to be suspended for sixty (60) days and is to be DISMISSED from the New York City Police Department; however, this penalty of dismissal will be held in abeyance pursuant to Section 14-115 (d) of the NYC Administrative Code for a period of one year, during which time the Respondent will remain on the force at the Police Commissioner's discretion and may be terminated at any time without a further hearing.



Raymond W. Kelly
Police Commissioner



POLICE DEPARTMENT

July 15, 2009

MEMORANDUM FOR: Police Commissioner

RE: Police Officer Rafael Montanez
Tax Registry No. 918022
107 Precinct
Disciplinary Case Nos. 83572/08, 84579/08 & 84986/09

The above-named member of the Department appeared before me on June 26 and June 29, 2009, charged with the following:

Disciplinary Case No. 83572/08

This case was calendared and scheduled for trial. At outset of this trial, however, the Assistant Department Advocate informed this Court that the charges and specifications were never served on the Respondent. Consequently, the Department conceded that since the charges were not timely served this case is time barred from prosecution by the Statute of Limitations. This Court, therefore, deems this case DISMISSED.

Disciplinary Case No. 84579/08

1. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on or about eight (8) occasions between about July 28, 2008 and about September 5, 2008, while designated as "Chronic Sick – Category B," and while assigned on restricted duty status to perform a tour of duty on each occasion from 0700 hours through 1523 hours, did fail and neglect to perform said assigned tours, after having been found fit by Department Medical Division physicians and directed, on or about the day preceding each such occasion, to report to work, in that said Police Officer, on each such occasion, reported sick during said assigned tour or within a few hours of the commencement of the following assigned tour, claiming sickness due to the existing condition reviewed and evaluated the previous day by said Department Medical Division physicians. (As amended at trial.)

PG 203-05 Page 1, Paragraph 1 – PERFORMANCE ON DUTY – GENERAL REGULATIONS

COURTESY • PROFESSIONALISM • RESPECT

2. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on July 28, 2008, having been found fit for restricted duty and ordered by Department Orthopedist Arnold Goldman, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

PG 203-03, Page 1, Paragraph 2 – COMPLIANCE WITH ORDERS – GENERAL REGULATIONS

3. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on August 6, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

PG 203-03, Page 1, Paragraph 2 – COMPLIANCE WITH ORDERS – GENERAL REGULATIONS

4. Said Police Officer Rafael Montanez, while assigned to 107th Precinct, on August 15, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D. to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

PG 203-03 Page 1, Paragraph 2 – COMPLIANCE WITH ORDERS- GENERAL REGULATIONS

5. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on August 29, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

PG 203-03 Page 1, Paragraph 2 – COMPLIANCE WITH ORDERS – GENERAL REGULATIONS

6. Said Police Officer Rafael Montanez, while assigned to the 107th Precinct, on September 2, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D., to report to work to perform his next scheduled tour of duty, wrongfully did fail and neglect to comply with said order.

PG 203-03 Page 1, Paragraph 2 – COMPLIANCE WITH ORDERS – GENERAL REGULATIONS

Disciplinary Case No. 84986/09

1. Said Police Officer Rafael Montanez, while assigned to the 107 Precinct, on or about August 14, 2008, wrongfully did secure his 107 Precinct Department locker with a combination lock displaying the serial number of said lock.

PG 203-03 Page 1, Paragraph 6 – COMPLIANCE WITH ORDERS – GENERAL
REGULATIONS

The Department was represented by David Green, Esq. Department Advocate's Office and the Respondent was represented by Craig Hayes, Esq.

The Respondent, through his counsel, entered a plea of Not Guilty to the subject charge. A stenographic transcript of the trial record has been prepared and is available for the Police Commissioner's review.

DECISION

The Respondent is found Guilty as charged.

Introduction

The Respondent was injured during a line-of-duty incident in 2001.¹ The Respondent submitted an application for Accident Disability Retirement on August 11, 2008 and an application for Ordinary Disability Retirement was subsequently submitted by the Department.² On May 12, 2009, the Medical Board Police Pension Fund Article II (Article II Board) drafted a memorandum (also referred to by Department as a '49') detailing the history of the Respondent's injury and its findings with regard to the Respondent's application for disability retirement. (RX A)

¹ The Respondent describes that incident during his testimony.

² Accident Disability Retirement is a pension consisting of three quarters pay and Ordinary Disability Retirement is a pension consisting of half pay.

After detailing the various incidents contributing to the Respondent's injury including lifting a five gallon bottle of liquid, playing with his son, and the various medical treatments including his surgery in April 2008, the Article II Board made a determination in the final paragraph of the report. They stated that: "The Article II Medical Board finds that officer is disabled because of the presence of a C5-6 artificial disc placement and continual radicular signs in the upper extremity; however, the Medical Board does not feel that the herniated disc is the result of the 2001 incident. This was demonstrated by the complaints and treatment of the officer following the line of duty injury and the change in the MRI after 2007 and the freshness of the pathological material found in the disc space. Therefore, the Article II Medical Board recommends approval of the Police Commissioner's application for Ordinary Disability Retirement and disapproval of the officer's own application for Accident Disability Retirement...." (RX A)

SUMMARY OF EVIDENCE PRESENTED

The Department's Case

The Department called Lieutenant Dominick Valenti, Doctor Arnold Goldman and Doctor Peter Galvin. It should be noted that witnesses appeared and testified in a different order than the above indicates to accommodate their schedules. This opinion, however, will list their testimonies as indicated for purposes of clarity.

Lieutenant Dominick Valenti

Valenti, a twenty-four year veteran of the Department, is the commanding officer of the Medical Division Absence Control Investigation Unit, a position he has held since

September of 2000. His duties include monitoring officers who are chronic sick, visiting those officers, and conducting investigations on officers who are violating the Department's sick procedures. Valenti testified that he has participated in such an investigation involving the Respondent, whom he has known for "probably at least 7 years," and that the Respondent has been designated chronic sick for a good part of his career.

Valenti informed the Court that at least twice during the time that he has known the Respondent he has had conversations with the Respondent with respect to his chronic sick status, letting the Respondent know that he was concerned about the sheer number of times he was reporting sick. According to Valenti, they also discussed the fact that the Respondent would have to demonstrate that he was going to come to work, and that he could face disciplinary action and/or suspension if he continued to report sick. In response, the Respondent would go over what his injuries were and the fact that he had pain.

When asked about the length of the tour that the Respondent would generally be assigned to while on a restricted duty, Valenti testified that such tours are typically from 0700 to 1523 hours, and that he believed the Respondent's regular days off (RDO) were on Tuesdays and Wednesdays. Prior to the surgery³ in April of 2008, the Respondent would have been on a 0700 to 1540 tour.

Describing the steps that the Respondent would need to go through in order to request a sick day prior to reporting to work, Valenti mentioned that the general policy is to report sick two hours before the start of the officer's tour of duty. Acknowledging that this is not a strictly enforced rule, he noted that even so, an officer would still have to call

³ Doctor Galvin and the Respondent subsequently testified as to the nature of the Respondent's surgery.

his or her command and request to be put on sick report. Those requests are automatically granted, and as soon as the officer calls the sick desk they are entered on the computer as being on sick report.

Valenti explained that if someone in Chronic B sick status would make that phone call on any Monday through Friday, the officer would be directed to the Special Medical District Surgeon, located at One Lefrak City (in this case Dr. Galvin), assuming it was open. If the officer was already at work and wasn't feeling well, he or she would tell a supervisor that they desired to report sick, and then report to the Medical Division if the clinic was still open. Most days, Galvin's office hours at the Special Medical Division are from 7:30 AM to 10 AM. If the officer reported sick at such a time that he or she would not have an opportunity to see Galvin that day, they would order the person to go back home and report to him the next day. There is no surgeon assigned to the Special Medical Division on the weekends, and so anyone reporting sick later on in the day on Friday, Saturday or Sunday would be expected to report to the regular weekend surgeon on rotation at One Lefrak between 10 AM and 12 PM. Furthermore, Valenti testified that if an officer reports sick on the day before his RDO and does not have a chance to see the surgeon that day, he or she will be asked to report to the surgeon on his or her RDO, but would not be returned to work that day.

When asked to discuss the Respondent's sick record generally, Valenti told the Court that the Respondent has been Chronic B from 2001 through present, with the exception of perhaps a six-month period within that time-frame. Prior to that time he had been designated as Chronic A starting sometime in 1997 or 1998. (A Chronic A designation is made if someone reports sick four or five times in a twelve month period.

A sixth event will earn an officer a Chronic B designation. A Chronic B designation can also occur if the officer is out sick a total of more than forty days in a twelve month period, regardless of the number of events.) According to Valenti, the Respondent reported sick twice in 2007 and twenty-two times in 2008. When asked to approximate how many days in total the Respondent was out for, Valenti stated that he had never added the number of days, but that one of the sick events related to the Respondent's surgery was, "certainly three to four months," and that generally the other sick events were, "probably a couple of days each."

When asked to examine the case folder, Valenti acknowledged that the Respondent had seen Galvin on Wednesday August 6, 2008, which was one of his RDO. The Respondent had reported sick on Monday August 4, 2008, about half an hour into his shift, but had not seen the doctor that day. Valenti pointed out that there was an indication made in the Respondent's sick chart that at the time in question the Respondent had been getting epidural injections and so the nurse had probably excused him for a day or two.

Valenti testified that Galvin had told the Respondent to report back to restricted duty work as of 1523 hours on August 8, 2008, a Friday, which would essentially mean that he would not be responsible to report back to work until the August 9, 2008, if that was a tour that he was working. He further stated that the Respondent reported for work on August 9, 2008, and at 1018 hours reported sick. As that time was beyond the clinic hours, he would have to wait at least until the next day to be evaluated.

Valenti testified that a review of a, "chronological listing of the dates that the Respondent went sick and what doctor he saw on those dates" indicated that the

Respondent had in fact seen a Dr. Lichtenstein on August 11, 2008 and was directed to return to duty on the August 11, 2008 as of 1523 hours.

Valenti stated that the Respondent next reported sick on Friday August 15, 2008 at 0638 hours, prior to the start of his tour. The Respondent was able to see Galvin that same day, and Galvin returned him to duty on Saturday August 16, 2008 at 1523 hours. The Respondent reported to work for his next tour, on Sunday August 17, 2008 and reported sick at 1313 hours. Valenti noted that it was too late for the Respondent to see the surgeon that day, and so he saw Goldman, an orthopedist, the next day on Monday the August 18, 2008. Goldman returned him to work as of 1523 hours on August 18, 2008. The Respondent had Tuesdays and Wednesdays as RDO, and did not have to return to work until Thursday August 21, 2008. Valenti did not know if the Respondent reported for work on the August 21, 2008.

Valenti further testified that on Friday August 22, 2008, the Respondent reported sick at 0630 hours, before the start of his tour. He saw Lichtenstein that morning, and was directed to return to work as of Monday August 25, 2008 at 1533. Valenti added that the Respondent had Tuesdays and Wednesdays as his RDO, and the next time that he had to go back to work was on that Thursday, August 28, 2008. Valenti did not know if the Respondent reported for work on the August 28, 2008.

On Friday the August 29, 2008, he reported sick at 0711 hours. According to Valenti, the Respondent saw Galvin that morning and was returned to work as of the end of his tour that day. He reported to work for his next tour, on Saturday, and reported sick at 1110 hours. Valenti acknowledged that at that point it would be unlikely for the supervisor at the sick desk to note that an officer was out sick in time to get them to see

the surgeon before at least Sunday, although the Respondent did not end up seeing Galvin until Tuesday September 2. (It was pointed out to the Court by the Assistant Department Advocate that the weekend prior to Tuesday was Labor Day weekend.) Valenti told this Court that Galvin returned the Respondent to work at 0845 hours on September 2, while he was RDO so he did not have to report back to work until Thursday September 4.

On Thursday September 4 the Respondent reported sick, although Valenti did not know whether or not he had gone to work that day. On Friday September 5, the Respondent again reported sick, at 0648 hours, and again saw Galvin. Valenti did not know whether or not the Respondent had attended physical therapy on the days that he had actually reported to work.

In response to a question from the Court as to what, if anything, he had concluded from the Respondent calling in sick and going to the District Surgeon, Valenti commented that he "concluded that he [the Respondent] was not going to come back to work." When asked to explain, Valenti stated that from the point that the Respondent returned from surgery, there were numerous occasions where he was immediately reporting sick after he was returned by the surgeon to restricted duty.

Valenti, regarding the times that the Respondent had called in sick, concluded that "the times that are either say 9:30 through 12:00 are all going to be after the clinic hours are over, so if the supervisor wanted to order him in to see the surgeon that day we would be unable to do so." He added that, "Generally when an officer is assigned to the Special Medical District is reporting sick on a Friday or a Saturday or a Sunday they are going to be left out sick until Monday to see the Special Medical District surgeon, so someone reporting sick in the Special Medical District on the weekend will automatically get at

least two or more days out sick.” He also acknowledged that as the Respondent had Tuesday and Wednesday RDO, it was almost a guarantee that he would be on sick report a minimum of three days, since even though in theory someone reporting sick on a Saturday could go see the surgeon on Sunday, the surgeons on the weekend do not have access to the Special Medical District folders, and so they are not as familiar with the officer’s situation and will generally defer to the Special Medical District surgeon on Monday instead of returning someone to work who is on Special Medical District from sick report on the weekend.

On cross-examination, Valenti acknowledged that on Saturday August 30 the Respondent had worked for about four hours before he called in and went sick, and that on August 17 he had worked for 6 hours and thirteen minutes, about seventy-five percent of his tour, before he called in sick. Valenti acknowledged that the Respondent has no power to control the times when the surgeons’ work, and that if the Respondent reports to work and then feels sick several hours later, the fact that the surgeon is not going to be there is out of his control, but pointed out that the Respondent is well aware of the surgeon’s hours.

When offered the following hypothetical question; “if someone calls in sick fifty times in one year and otherwise complies with all the rules in the Patrol Guide as far as being home when they are supposed to be and calling in and telling the right person, is that in and of itself misconduct calling in sick fifty times?” Valenti responded in the affirmative, stating that, “If you called in sick fifty times a years you would get suspended because you are obviously refusing to go to work.” When asked if, “just being sick a large number of times in one year by itself without violating any of the rules in the

Patrol Guide is misconduct,” Valenti responded, “No.” Valenti also acknowledged the fact that every single time Galvin returned the Respondent to work restricted duty, he did report to work.

Valenti testified that he was aware of the Respondent’s medical history, the nature of the Respondent’s injury, and the types of limitations that such an injury could place on the Respondent in the course of doing his job. Valenti acknowledged that sometimes there are violent incidents within precincts where officers have to take police action, and that it could happen that even an officer on restricted duty would become involved in such an incident. When asked if he had spoken to the doctors about the effects of painkillers in this or any other case, and how they could effect an officer’s ability to stay alert on the job, Valenti stated that he was aware of the side effects of painkillers such as Percocet, and that the “medication generally state do not operate a motor vehicle.” When asked if it would be appropriate for an officer to work in the precinct while under the influence of a painkiller such as Percocet, Valenti told the court that it would depend on how many he took, what the dosage was, and what effect it would have on his ability to perform his functions.

Valenti agreed that the Chronic B designation is governed by Patrol Guide section 205-45, which states in relevant part that “Category B members fit for duty after examination by the Special Medical District surgeon and who report sick again for the same reason prior to the next scheduled tour duty may be suspended without pay. If upon subsequent examination by the surgeon no objective findings of such illness or injury are found charges and specifications will be prepared by the commanding officer of the Medical Division.” Valenti also acknowledged that if an officer’s condition worsens the

day after they are put back to duty, even a Chronic B officer can go sick without committing misconduct. When informed that the Respondent felt that on the days in question his condition had worsened since he last saw the doctor, and that he was in too much pain to continue working, Valenti stated that he still believed the Respondent was committing misconduct based on the sheer number of events, and that in his opinion there was misconduct, "because in a relatively short period of time he is seen frequently by the surgeon and frequently returned and his efforts to go sick in his tour were just to circumvent the chronic sick procedure and avoid suspension and that's why he is going in three to four hours into his tour." Valenti agreed that the Respondent has not been charged with any charge or specification alleging violation of section 205-45 or any of the Patrol Guide provisions that govern sick leave, but reiterated that in his opinion he was circumventing that by going to work for three hours.

When asked if there was a discrepancy between what he had said before, namely that a Chronic B officer could be sent to the surgeon and returned to duty, then feel worse the next day and go sick again without committing misconduct, and the fact that the Respondent was claiming to have done just that, Valenti responded that, "There's a lot of different variables and even in [the Respondent's] case, when he violated the procedure back in April he wasn't suspended on that date, so when you repeatedly keep doing that or repeatedly circumvent the procedure, yes, you will get suspended. Nobody gets suspended the first time they violate the procedure."

Valenti testified that he had not actually told the Respondent that if he gets found fit for restricted duty, goes to work, stays a few hours, and then leaves he could get suspended, but that he had instructed him about the frequency of his sick events. He also

mentioned that the Respondent was previously instructed in 2004 about violating the chronic sick procedure. Valenti acknowledged that nowhere in the provision does it state that if a Chronic B officer is returned to duty status, reports to his tour and then leaves after four hours that he is in violation of that provision. Valenti noted that he did tell the Respondent that his frequent sick events could be viewed as malingering and that he could be suspended, but that the Respondent was never charged with malingering.

Valenti testified that when an officer is designated Chronic B, they are given a memo to sign that details the rules for those on Chronic B status, in order to give them fair warning of what is expected of them so that they can avoid any disciplinary action. He agreed that his conversations with the Respondent, which could have been in August of 2008, were essentially warning him of misconduct similar to this memo warning Chronic B officers about what conduct is expected of them and what could happen to them. Valenti acknowledged that he had not put those instructions in writing, nor did he have the conversation taped, but told the Court that he is not required to give anyone a written or verbal instruction.

On re-direct examination, Valenti agreed that in a situation like the theoretical scenario previously presented, involving a member on restricted duty being present at the precinct when something goes wrong and having to take some police action, there might also be civilian members present in the precinct at that time. Valenti stated that he did not believe that the civilian members would be expected to take police action, but that they could certainly be involved in the incident. He also testified that someone on restricted duty may not be able to take police action.

Doctor Arnold Goldman

Goldman has been working as a Department orthopedic surgeon at 1 Lefrak City for the past 16 years. He graduated college at Poly Tech of Brooklyn and went to medical school at State University of New York. He did an internship at Beth Israel Orthopedic, a residency at NYU Bellevue, and 2 fellowships at NYU Bellevue. He has been practicing as an orthopedic surgeon since 1981.

Goldman was shown a an orthopedic consultation Department Exhibit (DX 6) that he had filled out and signed in response to Dr. Galvin's request for him to examine the Respondent. The request read "status post surgical disc replacement 4/30/08 start PT when is restricted duty reason." (DX 6) Goldman explained that he was being asked to evaluate the officer as to whether it would be reasonable post operation for the individual to go back to work, and if so, when. Goldman explained that a Department surgeon who is not an orthopedist may make such a request for a variety of reasons, including needing treatment or advice. Goldman told the Court that he saw the Respondent on July 8, 2008 at 1 Lefrak City.

Goldman testified that he is familiar with the type of surgery that the Respondent had undergone, as well as the procedures and post operative procedures involved in such a surgery. Goldman told the Court that when Galvin requests this kind of an opinion the member of service he is being asked to see usually has two medical folders, an orthopedic folder and a Special Medical Division folder. When asked which, if any, materials he would look at before examining someone like the Respondent, Goldman replied that usually he looks at both folders, determines whether surgery was performed, looks at past notes, looks at physical therapy notes, and asks the officer if he has any notes to show

him from his doctors, therapists, any MRIs, X-rays, or CAT scan results. Goldman had no independent recollection as to whether or not such notes were provided by the Respondent on that day.

When asked to describe the evaluation that he made of the Respondent during that visit, Goldman replied that he had documented that the Respondent had had a disc replacement surgery, that he was on several medications, that he was able to sit in a car to get to the examination that day, and that the wound at the base of his neck was well healed. Goldman further testified that he had documented that overall he had felt that the Respondent's C spine and neck motion were limited, that post operatively the Respondent was able to bend his neck up and down and from side to side, that the Respondent goes to therapy, and that there was no antalgic gait (which Goldman explained means, "a limp, to and from the office.").

When asked which medications he had indicated that the Respondent had been on, Goldman responded that he had indicated Zanaflex and Percocet. He was unsure if that information had been in the chart or if he had asked the Respondent at that time, and was also unsure if the Respondent had actually taken the medication that day before the examination. He stated that he was familiar with both medications and that the Respondent was also on Lyrica, and told the Court that the Respondent had overall good range of motion on both upper extremities.

Goldman testified that his purpose in making this examination was to evaluate whether or not the Respondent could go back to restricted duty. He told the Court that he had also written that he needed some more documentation from the Respondent. When asked what documentation he had been looking for, Goldman responded that it was most

likely the operative and post-operative notes from the doctor, which would contain information on, "what surgery was performed, what prosthesis or hardware would be used. I get a sense of how long the operation would be, what they found. If it was a back or neck operation it would be how much disc material or how much arthritis was entailed." When asked how this type of information would affect his prognosis and decision, Goldman answered that for a more technical or lengthy procedure he might delay the individual going back to work, and that he would want to know if there was an infection or a blood clot.

When asked if he had made any notes regarding any pain that the Respondent may have been in on that date, Goldman told the Court that he had not documented anything. He further stated that the fact that an individual is in pain is not prohibitive of that individual being returned to restrictive duty, since it, "is not uncommon for somebody to have pain several months after surgery. Even from various sprains, aches, anyone could have pain and be able to do sitting work similar that he is doing at home. That is my determination if that individual could do that." Describing the steps he takes to evaluate the amount of pain that an individual is in, Goldman told the court that, "I observe, go through the notes, examine them, see how they get to and from the office, see how they get physically to and from Health Services Division, and then try to put everything together with documentation, speaking to other doctors and make a determination." Goldman stated that he had made notes about how the Respondent had gotten to the facility that day (he was "able to sit in the car"), and about the fact that the Respondent was "able to go to physical therapy two or three times a week." When asked if these notes played a role in his decision, Goldman told the court that, "He was able to

sit in the car and able to go to therapy two or three times a week, and I think he should be able to go to work and do sitting type of work. That is usually a good sign.” Goldman further stated that he had recommended more physical therapy and that it was his opinion that the Respondent should be able to return to restricted duty at the end of July, 2008. Goldman said that he considered that length of time (three months post operation) appropriate based on his experience with over one thousand cases involving neck and back surgeries.

Having been shown another document, Goldman identified it as a sick report or return form for an evaluation he performed on the Respondent on July 28, 2008. (DX 8) Goldman told the court that he had written the following on the report: “On Percocet, Lyrica, Zanaflex, tried to come here today positive straight leg raising (Goldman explained that this is an examination exercise). Goes to physical therapy two or three times a week, gets driven there. Drove here today.” Goldman told the court that the diagnosis was “low back pain, neck pain post op,” and that he had also put down that the Respondent could return to limited duty on August 3, 2008 at 2400 (that time was corrected to 1540).

When asked why he had seen the Respondent that day, Goldman told the court that he may have been covering for the Special Medical District that day. “It may have been a date that one of the other doctors was away, the individual came in late, or for whatever reason. It’s not uncommon for me to ask to see these individuals as well.” Goldman stated that such an examination takes approximately five or ten minutes, and reiterated that he usually reviews the restricted folder and the orthopedic folder, which should have had his previous notes in it, although he had no independent recollection of

having done so on this date. Goldman also stated that he had no independent recollection of anything that the Respondent may have told him on this date (other than what was in the notes), but testified that if the Respondent had said something that was unusual or new that he (Goldman) was unfamiliar with, he would have documented that information.

When asked on what criteria he had based his determination that the Respondent was fit to return to restricted duty, Goldman responded that it was, "Based on the fact that he was still able to go to physical therapy, sit in the car to get to Lefrak. I was aware that he was on the same medication, if I would have seen anything significant I would have documented that, but it was a similar examination that I had seen before." Goldman said that although he saw the Respondent on Monday, July 28, 2008, he only returned him to duty as of the end of his tour on Sunday, August 3, 2008, essentially giving him a week off to "continue with therapy for the rest of the week, if I asked to see him usually if he still had pain post op he was approaching three months I figured let him start and keep him out the rest of the week." Goldman told the court that he generally tries to give officers the benefit of the doubt if they are having pain, and let them stay out the rest of the week. Goldman further stated that he usually tells the officer what he is doing, and that if the officer then says something of any significance in response to his determination, he would usually document those statements. He also said that, "if he wants me to examine him I will do so, if he wants me to examine him with one of the supervisors, whatever the officer wants me to do. I try to accommodate the officer." Goldman told the Court that seeing as there was no documentation that anything like that had happened on that day, it was fair to say that there was no request for further examination.

Upon cross-examination, Goldman was asked if he knew why the Respondent had had this surgery done. He responded that it is usually performed for the general arthritis or herniated disc at a particular level. When asked to explain what a disc does in general and how a herniated disc could cause pain and discomfort, Goldman stated that a herniated disc is, "like a jelly doughnut, where the inside jelly can push out through the rest of the doughnut and push on the nerves. When it pushes on the nerves it irritates the conductivity going through the muscle, so it can cause pain and after that possible weakness in the extremity." He acknowledged that depending on how much it bulges out it can cause certain levels of pain, and that this type of surgery, which he would classify as a serious or major surgery, is used to remove that disc and put in some sort of prosthetic. He told the Court that right after the surgery there would be a great deal of pain, and that having the surgery does affect a person's range of motion afterwards.

Goldman acknowledged that the act of surgery has the effect of trauma to the patient's body, and that in the post operative state scar tissue could develop in the surgical area which could potentially remain in the body. Goldman agreed that one of the purposes of physical therapy is to try to eliminate the scar tissue, but that sometimes, despite a person's best efforts, scar tissue can remain which can inhibit range of motion in the bones and muscle, and potentially cause pain by infringing on a nerve area of the body. Goldman acknowledged that there are times in the post operative state, even a year later, where someone can have great days and then all of a sudden have a setback where they have moderate to severe pain mere hours after they felt great. He agreed that having scar tissue in the affected area could cause such a situation. Goldman acknowledged that driving for a long distance could potentially aggravate an injury such as the

Respondent's, as could the Respondent having exerted himself too much during physical therapy. He also acknowledged that there is a level of pain so severe that it could keep someone from being placed on restricted duty, and that that is the reason, along with the range of motion, that an officer who has undergone this kind of surgery is not put back to work for two or three weeks.

Goldman testified that his set schedule at Lefrak City calls for him to be there in the mornings on Mondays from 7:30 AM to 10:40 AM, on Tuesdays from 7:00 AM to 11:10 AM, and on Thursdays from 7:00 AM to 11:40 AM, but that if he has to stay later to see officers who are waiting, he does. Goldman told the court that although he did not remember specifically reviewing the notes in the Respondent's case, it would be his usual practice to do so, and that this review usually takes place with the officer or before they are seen, but not usually the day before. Goldman explained that if he is seeing an individual for the first time he may spend "ten, fifteen, twenty minutes going through first the notes, the operative notes, if it was somebody that I had seen before either five or ten minutes either post op or physical therapy."

Goldman testified that "99 percent" of cases he handles for the Department involve orthopedic patients. He acknowledged that he had indicated that he had performed a leg test on the Respondent during the July 28, 2008 examination, and that other tests he typically does for someone who is post op with a back injury include range of motion and sometimes checking their post operative reflexes and looking at their wounds. Goldman told the Court that he tries to do these types of tests every single time he examines someone for the purpose of determining whether or not they can go back to restricted duty, in order to determine both from objective and subjective points of view

whether an individual can do a certain type of work. He further stated that if someone was three-and-a-half or four months post-op and complaining of severe pain, he would observe the individual walking to and from the examination room, but that it is unusual and not really done to just walk in and speak to a patient and not examine him or her at all before making a determination as to whether or not they can perform restricted duty. He told the Court that he would perform a few of the tests that he had indicated to see how the patient's range of motion was, and talk to the individual in order to get a correct assessment as to his or her duty status and what he or she is capable of doing. Goldman acknowledged that he did not feel that he could get an accurate determination as to an officer's duty status after an injury such as the Respondent's, and when the officer is complaining of pain, by simply looking at the officer while they are sitting down in the examination room and speaking to them. He noted that aside from asking the officer to perform some of the physical tests described, he would also ask him how he got to physical therapy and other types of questions.

On redirect examination, Goldman was shown another document which he identified as a sick report that he had filled out on August 18, 2008 in reference to the Respondent. Goldman read the following notes from the report: "Well healed wound noted over base of right side of his neck today, clean and dry actually lower back feels worse than neck, overall externally rotate behind head, rotate bilaterally, able to go to physical therapy, either gets a ride neighbor or father-in-law drives him, okay for pain management as well as follow up X-rays." (DX 9) When asked, Goldman told the court that he had probably determined that the lower back felt worse by asking the Respondent. Goldman testified that as a result of his examination he had determined that the

Respondent should be able to go back to restricted duty as of August 18, 2008 at 1523, essentially the next tour of duty after the day that he saw him.

Upon re-cross examination, Goldman reiterated that before he made that determination as to what duty status the Respondent should be returned to, he made sure to actually give the Respondent a physical examination.

Doctor Peter Galvin

Galvin is a Department police surgeon at the Medical Division. Upon his graduation from New York Medical College, he did his internship and residence at St. Vincent's Hospital in Manhattan. He is board certified in internal medicine and has worked in private practice since 1983. In 1992, he joined the Department, and was originally assigned to the Jamaica clinic for a few months before being reassigned to Staten Island. After twelve years in Staten Island, Galvin was assigned to Queens (Lefrak City), and has been working there ever since. Currently, he is assigned to the Special Medical District; ordinarily the members of the service are assigned to medical districts based upon the zip code of their residence, however SMD is a district that is only for members of the service who are designated as Chronic sick.

Galvin testified that he knows the Respondent and that he has known him for quite a long time. He knew the Respondent when he was in Staten Island because anytime the Respondent would report sick he would need to come in to see the district surgeon (Galvin) in order to be evaluated for his ability to work. Galvin continued to see the Respondent after he became the district surgeon for the SMD because the Respondent was designated as Chronic B. Galvin told the court that when he saw the Respondent at

the SMD, his specific complaints usually revolved around back and neck pains, and that he (Galvin) was aware that the Respondent had undergone surgery to have a disc replaced in his cervical spine. He also explained that when he evaluates a member of the service in his capacity as a district surgeon, the purpose is to determine their ability to return to their duties, and that the determination could be based on a variety of things, including the physical examination, letters from treating physicians, evaluating Article II minutes (which contain a description of the officer's injury, how it occurred, and a determination of whether they are capable of performing full duty or are disabled), and the opinions of other Department specialists.

Galvin informed the Court that there are three levels of duty that he can assign an officer to; full duty, limited duty, and restricted duty. Limited duty is usually for a specific amount of time, up to six weeks, and then there is restricted duty, which is the same as limited duty only intended for a longer term. The type of duty that someone on limited or restricted duty would participate in would be determined by the person's command, but in general the person is not permitted to do patrol or to drive a Department vehicle, and is instead assigned to some sort of administrative duty within the confines of the command. Galvin noted that even if someone is deemed to be disabled by the NYPD Article II Medical Board and eligible for a disability retirement, they are usually still capable of working in a restricted capacity, and that even though the Respondent in this case has been approved for a disability retirement, that determination would not have anything to do with whether or not the Respondent can perform limited or restricted duty. Galvin explained that the duty the disability retirement speaks to is only full duty, and not restricted or limited duty.

Having been shown a sick report and an accompanying note from August 6, 2008 (DX 1), Galvin testified that they had been prepared in connection with his evaluating the Respondent on that day. According to Galvin, the Respondent had previously seen the Deputy Chief Surgeon Romanelli, on August 4, 2008, and had returned to the clinic on the August 6, 2008 for evaluation. The evaluation took place in the SMD office, and lasted approximately five minutes, after which Galvin returned the Respondent to restricted duty status. Galvin told the Court that he had included the following note in his evaluation: "August 6, 2008 0910 states back spasms, had epidural lumbosacral yesterday. States unable to sit or drive. On Percocet. Had ride to clinic. Continue physical therapy. Restricted duty August 8." (DX 1) Galvin did not recall any other conversation that he had had with the Respondent at that time.

Galvin told the Court that the reason he did not return the Respondent to duty until August 8, 2008 was in order to give him an extra several tours so that the discomfort from the epidural would subside. Galvin testified that he informed the Respondent that he was being returned to restricted duty, and explained that by doing so what he was telling the Respondent that, in his opinion, the Respondent was capable of working in a restricted capacity, although he was unsure from an administrative standpoint if such a an opinion is considered an order or a recommendation. Galvin stated that as a Department district surgeon he holds the uniform rank of Inspector.

Galvin was shown another sick report, dated August 15, 2008 (DX 2), and testified that he had prepared it in reference to the Respondent. Under "Initial Office Visit," the report read in relevant part: "Went to work and then physical therapy. History of one level cervical disc replacement April of 2008. Complains of neck and back pains.

Complains of pain with sitting. Was able to drive to the clinic and restricted duty August 16.” (DX 2)

Galvin testified that during the August 15, 2008 examination, the Respondent informed him that he had driven to Lefrak City. When asked if that fact played a role in his subsequent determination, Galvin stated that, “Well, if you are able to sit in traffic and drive for an hour approximately, you should be able to do work at a diminished capacity.” Galvin reiterated that the type of restricted duty to be performed in such a situation is at the direction of the particular command, but that the person is not being directed to stay trapped in a chair for the entire tour of duty, and could be doing any number of things within the confines of the command. Galvin told the Court that when making his determination he did take into consideration the fact that the Respondent complained of pain when sitting, but that he still felt the Respondent was capable of being returned to restricted duty and informed the Respondent of as much. Galvin also said that he believed the Respondent attended physical therapy somewhere in Staten Island.

Having been shown a third sick report, dated August 29, 2008 (DX 3), Galvin testified that he had filled it out in reference to the Respondent, and that the sick report contained, in relevant part, the following note: “Back pain. Went to work then physical therapy. Also neck pain. On Percocet/Zanaflex. Drove to clinic today. Restricted duty August 29.” Galvin told the court that the fact that the Respondent went to work and then physical therapy did not play a role in his decision to return the Respondent to restricted duty, whereas the fact that the Respondent was able to drive to Lefrak City did play a role in that decision. Galvin testified that if anything had changed in the Respondent’s

condition since the last time he had seen him, or if the Respondent had indicated anything of the kind, that is something that he would have made note of in the sick report, and that in fact the Respondent had made no such indication. Galvin noted that he had once again informed the Respondent that he was returning him to restricted duty as of the end of his tour on that same day, August 29, 2008.

Having been shown a fourth sick report dated September 2, 2008 (DX 4), Galvin told the Court that the top portion appeared to have been written by a weekend surgeon, and read in relevant part: "History of single level fusion C spine. Saw Dr. Galvin yesterday, pains very diffuse neck and back decrease range of motion lumbosacral spine. . . . He says hurts too much. See Dr. Galvin 9/2/08."

Galvin testified that he saw the Respondent on September 2, 2008, and having seen him, wrote the following addendum on the sick report: "Neck and back pain. Pain both shoulders. Went to work for a few hours on August 30. Drove to the clinic today. Restricted duty 9/2." (DX 4)

Galvin told the Court that he returned the Respondent to restricted duty as of that same day, but that the Respondent was on his RDO. He also stated that the fact that the Respondent had again driven to the clinic did play a factor in his decision, and acknowledged that the pain in both shoulders may have been a new complaint which is why he wrote it down, but that, as reflected in his writings, there were no other changes in the Respondent's condition that he was made aware of. Galvin told the Court that the examination lasted approximately five or ten minutes, after which he informed the Respondent of his decision.

Having been shown a fifth sick report relating to the Respondent, dated September 5, 2008 (DX 5), Galvin testified that he had written the following: "Neck and back pain. Drove to the clinic today. Did a full tour yesterday. Exam unchanged. Can do restricted duty." Galvin testified that as far as he could tell, and based on his evaluation of the Respondent, his condition did not change during the period of time from August 2008 through September 2008. Galvin agreed that on some occasions the Respondent reported for work after seeing him as directed, and on some occasions he did not.

Galvin acknowledged that neck and back pain are subjective things, and when asked how he had made his determination that the Respondent was fit for restricted duty if those pains are subjective, Galvin responded that the determinations were based on, "The fact that nothing had changed from the prior evaluation, based upon the evaluations of the Department orthopedist, based upon his information that he gave me he had not had any initial test, there was no information he could give me to indicate there was any change."

Upon cross-examination, Galvin went through a description of his medical background, starting with his internship at St. Vincent's. Galvin told the Court that it was a rotating internship which lasted one year (it was during 1980), and during which he participated in a few different disciplines, including internal medicine, radiology, orthopedics, and surgery. Galvin testified that following that internship he did his residence in internal medicine and no other discipline, and that he is board certified in both internal medicine and in quality assurance and utilization review medicine.

Galvin acknowledged that he did not have extensive training during his residency in orthopedics, orthopedic surgery, or in any type of surgery or neurology. He told the

Court that over the years he has taken one or two courses in that area that was for primary care practitioners, but that he has had no fellowship in that area, nor has he performed or assisted in any orthopedic surgeries that he can recall. Galvin was not entirely sure that he had seen an orthopedic surgery done. When asked what he would do in his private practice if someone came to him with a complaint of severe prolonged back pain, Galvin stated that he would perform a physical examination and then based upon that, "it depends. I might prescribe medication. I might send the patient for X-rays," but that he would not immediately refer the patient to a specialist. Galvin acknowledged that if he could not determine what was wrong with him, there would be times where he would feel that the best course of treatment for that patient would be to see an orthopedist or a neurologist, but did not agree with the blanket statement that a neurologist or an orthopedist is more qualified than he is to treat someone with that injury or problem.

Galvin testified that as a matter of course when examining an officer he looks at his medical records relating to treatment by his primary physician, specialist, and the like. He told the Court that although he couldn't remember the specifics of each of the Respondent's visits, it was "more than likely" that he did in fact do so for the Respondent as well. Having been shown the Medical Board's Article II minutes from the hearing involving the Respondent, prepared on May 12, 2009 (RX A), Galvin was asked to look at Paragraph 7 of the second page, which states that an MRI was performed on April 21, 2001 on the Respondent's back. Galvin was unable to recall if he had reviewed the finding of the MRI before examining the Respondent. When asked to explain the impression the document talked about, Galvin told the Court that it means that, "the person in question has a developmental problem with his spinal cord and has arthritis in

his spinal column.” Galvin stated that some of the findings were common and do not normally come from an injury; “They would be findings you find on an MRI of a person in their thirties.”

Galvin was then asked to read Paragraph 16 of the minutes, which states that the Respondent underwent a lumbar epidural steroid injection. Galvin acknowledged that he has never performed one of these injections, but that he does know the purpose of a steroid injection, namely to alleviate pain in the area by injecting the nerve root. He agreed it would not be proper use of this epidural to inject someone who is not experiencing severe pain, as the injections are usually done on the basis of the person’s complaints. Galvin was then asked to take note of Paragraph 18, which states that there was another MRI done, “cervical spine the impression being osteophytic ridge C5-6 level.” When asked to describe for the Court what that means, Galvin answered that an “osteophytic ridge is the equivalent of a bone spur,” and that this was on the lower part of the cervical spine, or the neck area. Galvin explained that, “a bone spur is an abnormal growth of bone that may cause discomfort or may impinge on a nerve or something that it shouldn’t be pressing on so it may cause discomfort.” Galvin was then directed to Paragraph 19, a note from Dr. Suarez which states that the officer’s cervical spine showed degenerative disc disease at C5-C6, which Galvin explained as “a form of aging or arthritis of the spine where the discs are beginning to get compressed,” and which could cause pain, and on occasion severe pain.

Galvin was directed to Paragraph 25, which describes another MRI and findings from November 19, 2007 detailing what Galvin told the Court was, in layman’s terms, “a slip disc, a disc that’s moved a little bit and no longer in alignment in the spinal column

so its sticking out.” Galvin stated that it would be hard to answer what problems a patient with this type of diagnosis could go through, since it would depend on a variety of factors that the findings do not discuss. Galvin was unable to recall whether or not he had looked at this report when he was making his determinations, and that it depended upon what was available in the Respondent’s folders, but stated that as a matter of course he would have wanted to look at the report.

Next Galvin was directed to Paragraph 28, which details the surgery that was performed on the Respondent on April 30, 2008. (The operative procedure was discectomy and arthrodesis of C5-6 microscopic decompression of nerve roots, C5-C6 and C5-C6 and total disc replacement using prestige artificial disc.) Galvin was unable to recall if he had reviewed the surgical notes and surgical records from that surgery, although he did state that it was, “more than likely” that he did. Galvin acknowledged that he has never performed this surgery nor has he treated anyone as follow-up after this surgery, nor can he recall ever having observed it. Galvin stated that he would not know the specifics of the surgery itself, but is familiar with the surgery and might have run across it a few times in his experience in the Department. He testified that he does know about the complications and the type of pain a person may feel after this surgery, and that his opinion in this case was based on the opinion of the Department orthopedist, who does have that kind of training. Galvin further stated that he was sure that there was an evaluation during that time by the orthopedist. He explained to the Court that a patient may have one medical folder or may have multiple medical folders, and that the folder contains the sick reports, copies of MRI reports, evaluations from treating physicians, physical therapy reports, and reports from Department specialists. Galvin testified that he

would not return to work someone who had undergone an orthopedic procedure until they had been evaluated by an orthopedist to make that determination. Galvin could not recall if on the dates in question in the Respondent's case he had ever brought in the orthopedist to see what his opinion was, although he pointed out to the Court that, as noted in the record on September 5, 2008, he had the Respondent see Dr. Miller, the Department orthopedist, because the officer had "gone sick so many times I thought maybe I'm missing something." Galvin testified that he had asked for Miller's opinion, and that Miller's opinion was that the Respondent was capable of doing restricted duty. (DX 7) Galvin acknowledged that he had not sought Miller's opinion on every occasion, nor had he examined the Respondent on every occasion that he found him fit for restricted duty.

When asked if he thought it was prudent to send someone back to restricted duty without examining them, Galvin told the Court that, "if I'm seeing the person two or three times a week for the same problem and he is telling me nothing has changed, then I think it would be reasonable to assume that the examination wouldn't change," and reiterated that in September 2008 he felt that it was probably reasonable to seek out another orthopedic opinion, because the Respondent had gone sick so many times that he thought to be perfectly fair he may have missed something. Galvin testified that he is aware of the types of tests a neurologist or orthopedist can perform on anyone to determine the cause of pain, but that on several occasions he decided not to do any of them.

Galvin acknowledged that pain is a subjective thing, and that driving a long distance could potentially aggravate the injuries of someone in the Respondent's position, as could sleeping in an awkward position. Galvin agreed that there were occasion when

there would be a time frame of 24 to 36 hours between the time that he made his determinations that the Respondent could return to work and the time that the Respondent would actually have to do so, and that the level of pain the Respondent may have been feeling during their meetings might have been a different level of pain than the Respondent felt a day later. Galvin acknowledged that he did not really know the level of pain that the Respondent was in on those days when the Respondent worked a few hours and then left, nor what kind of limitations that pain was putting on the Respondent's ability to work.

Upon re-direct examination, Galvin was reminded of the line of questioning as to whether or not he had requested that a Department orthopedist examine the Respondent, and was then shown a document that he identified as an orthopedic consult from Dr. Goldman, a Department orthopedist. (DX 6) Galvin testified that he was the one who had requested that the orthopedic consultation take place, as indicated by his name and signature on the document. Galvin testified that his own notes on the document read as follows: "Status post cervical disc replacement on April 30, 2008. Started physical therapy. When is restricted duty reasonable?" Galvin further testified that the Department orthopedist recommended restricted duty by the end of the month (July 2008), when the Respondent would be "three months post-op." Galvin told the Court the Respondent was returned to restricted duty sometime in July 2008 although he did not recall the exact date offhand.

Galvin was shown another orthopedic consult which he identified as the orthopedic second opinion consultation by Miller that he had made reference to during the cross-examination. (DX 7) Galvin testified that the document was dated September

5, 2008, and that he had made the request that same day. Galvin told the Court that the document contains the following notes, which he had written; 'Cervical discectomy on April 2008, one level. Complaining of neck and back pain, able to drive,' and that in asking for the consult Galvin was asking Miller for a second opinion as to whether this particular person was fit for restricted duty. Galvin told the Court that the request was prompted by the multiple sick occurrences and multiple complaints that the officer was having, "so I said let me get a second opinion because maybe we missed something the first time around." Galvin further testified that Miller saw the Respondent on September 5, 2008, and wrote the following notes on the document: "MOS seen in follow up. Neck and back pain on physical therapy three times a week intermittent, able to drive, pain radiates looks like buttocks, has had series of four injections for back without benefit, on Percocet twice a day, moderate to severe [here Galvin indicated that there was an arrow down on the left side of the document, possibly indicating a decrease]. 65 to 70 degree FF [Galvin was unsure what FF stood for]. 75 to 80 degree abduction arms bilaterally. No atrophy. Decreased grip bilaterally, right greater than left, slow stiff gait without external support, 15 degrees active flexion, 10, 15 degrees extension L spine, difficulty attempting to ambulate on toes, bilaterally calves equal in girth, no atrophy." Galvin also testified that Miller had checked restricted duty on the document, indicating that he felt that the MOS was capable of performing restricted duty, and confirming Galvin's own determination made on that same day.

Galvin told the Court that the fact that a member of the service is in pain does not prohibit that member from reporting to work, generally, and that the fact that an

individual complains of pain to a district surgeon does not in and of itself indicate to the surgeon that the member of service is incapable of performing restricted duty.

Upon re-cross examination, Galvin acknowledged that although pain is subjective there is potentially a certain level of pain that would prevent someone from going to work or from working their full tour. Galvin acknowledged that in the place where he works there are not usually violent situations that call for police action, while such incidents do occur inside police precincts. Galvin further acknowledged that a certain level of pain could potentially prevent someone from defending themselves in one of those situations.

Galvin testified that the side effects of Percocet are sleepiness, drowsiness, and potentially dizziness, and that it would usually not be a good idea for someone to drive while on Percocet. Galvin acknowledged that if part of someone's job is sitting at a desk, Percocet could potentially cause them to fall asleep.

The Respondent's Case

The Respondent testified on his own behalf.

The Respondent

The Respondent is a 14-year member of the Department currently assigned to the 107 Precinct. Prior to joining the Department he served time in the military performing four years active duty and then reserve time. Up until 2001 he worked in the 70 Precinct, which was prior to the injury to his back and neck. Prior to his injury he stated that he had made approximately 50 arrests.

On January 29, 2001, the Respondent had an incident which resulted in the injury to his back and neck. He stated that he was working in Brooklyn Central Booking at 120 Schermerhorn Street where "we were bringing prisoners up to the mezzanine level, it's a level above the judges in the back of the judge's bench and we had a prisoner get extremely violent in the cell refusing to go into the cell and to make it brief I was able to go - - they were able to distract him. I got behind him to restrain him and his right arm broke free and he began to strike me in the face and neck with his free arm. Ultimately when the officers saw me ducking taking the strikes I didn't see it because I was, of course, ducking, several of them tackled the both of us and I went backwards on the concrete wall and onto the ground." He stated that when he went back into the concrete wall his neck and his back made contact with the wall.

He further stated that after the incident he "first thought I was fine then my back went into full burn. I mean that my entire back was on fire. I was in a lot of pain from my neck to my buttocks. I was removed I think to Methodist Hospital."

The Respondent stated that as a result of this injury the Department kept him out of work for several weeks to see the Department's orthopedists and then he had authorization from the Department's orthopedists to see his personal doctor. When asked what type of diagnosis he received he replied "the first day that it happened it was listed as soft tissue trauma to the neck and back and then the orthopedic doctors proceeded to tell me you know, you are just having spasms you received trauma to your back. Just put me on Percocet or Vicodin and told me to just relax. Keep your legs elevated. Go to physical therapy." The Respondent further stated that between the years 2001 to 2006 he thought he was improving at times to the point where he thought he could continue his

career but then he got re-injured. He stated that the first time he injured his back he believes it was in 2003 but he did not recall exactly what happened. He explained "I am not sure, I think it might have been off-duty. I am not really sure." He further stated that he re-injured his back and neck on the job when he fell into a hole running to a burglary explaining that these are the things that happen when you are on patrol.

The Respondent testified that prior to having his surgery he "was in a significant amount of pain with my neck and pain going to my bicep areas, some numbness in my left hand, sciatica, sporadic sciatica it wasn't constant, my lower back, my mid-back is constantly in spasms now and just the core of my lower spine is in a lot of pain also." He stated that he went to physical therapy two to three times a week and received electric stimulation heat and deep tissue massages. He stated that he also took medications consisting of Vicodin and "then it was elevated when they saw the Vicodin wasn't working they prescribed Percocet and then they prescribed OxyContin."

The Respondent stated at one point he decided to actually have the surgery to repair his back because he was "constant pain for maybe six months and I had done the consultation with the neurologist and he told me that you definitely need help with the situation with my back that he told me that the neck area was affecting my mid-back and my problems with breathing, I was having a lot sharp pain breathing." In April of 2008, the Respondent underwent surgery and after the surgery he began to see Dr. Goldman, who rendered his evaluations in early July of 2008. When asked if he could remember anything about the evaluation with Goldman the Respondent stated that, "I remember responding to the clinic and it was quite empty and I was told by an officer who was in the office that Goldman was waiting for me and I walked into his office he was already

typing up the form, approximately halfway through it. I walked in to about the center of the room and I believe he had me do I think a couple of raises with my arms at most and asked me to walk back to the door and that was it. He asked me if I needed any authorizations and he told me to wait outside.” The Respondent stated that he was then placed on restricted duty after his surgery.

The Respondent stated that when he was placed on restricted duty he was working at the 107 Precinct. He said at that time he was living in Staten Island and the commute from Staten Island to the 107 Precinct took “an hour plus.” When asked, the Respondent told the Court that the tasks he was asked to perform at the 107 Precinct, “were to answer phones, act as an assistant desk officer, T/S operator, a lot of reaching involved, a lot of lifting up the phone. Anything to do with my arms was painful they were asking me to fingerprint and there were days that I just couldn’t do it and I would tell them that they have to find someone else because of leaning and everything.” He stated that his days off were the only days he could take his medicine and just elevate his feet and try to relax with the hope that the spasms would go away. He stated that he did not participate in any activities, he would just stay home on those days. When asked what type of pain he was experiencing he stated “moderate pain, it wasn’t like when I first got re-injured but I am still up there. I am still at a three or four. Right now my back and my neck is about two, three and this is, I can’t function like this.” He stated that painkillers would take the edge off some but they didn’t completely remove the pain. He also stated that the side effects from the painkillers were “grogginess, unable to concentrate, very sleepy.” The Respondent stated that he would not take painkillers and drive an automobile nor would he take painkillers to get through a day of work.

The Respondent further explained with regard to his condition that “there are days when I feel okay and then there are days when it is worse and then even worse, I mean horrible days also.” The Respondent told the Court that even on the days that he was feeling “okay,” that did not mean that he was pain free, just meant that the pain was tolerable. He added that when he was returned to restricted duty there were days when he went to work and was in pain. He acknowledged that despite the pain that he was in, he was able to complete his tour of duty. He also acknowledged that there were days when he woke up and would not go to work and would call in sick. He explained that, “those were the days when I woke up and I just knew that I would not be able to make it to work. I just knew I would not be able to function at work. There were some days when I couldn’t even walk I had to hop to the bathroom because one leg was having severe sciatica over the other. Those were just the days that I felt in my heart that I could not do it. There was no way I could do it.”

The Respondent denied that he ever took into account the day of the week when he decided to call in sick meaning that he would not deliberately call in sick the day before his days off, which were Tuesdays and Wednesdays. He further acknowledged that there were times he would call in sick on a Monday but denied that he called in sick on Monday knowing that he would have to go see the surgeon on his day off. He added that the reason why he called in sick was because, “he was in too much pain.” The Respondent was then reminded of the days when he was found fit for restricted duty, i.e. July 28, August 6, August 15, August 29 and September 2, 2008 when he was examined by Dr. Goldman and Dr. Galvin after calling in sick. He acknowledged that on those days he was found fit for restricted duty and he was informed by the doctor’s that “they

would tell me I would have to keep trying.” When asked what would an appointment with Galvin

be like he stated “he would announce my name on the loud speaker, tell me what number room he is in, I would walk in, I would tell him what occurred that day, whether it was during the drive in to work, at work, what occurred with my pain and I would stand there. He would offer me to sit, I would tell him I just sat in the car this whole time, there is no way I can sit down and he would tell me to keep trying; “I’m putting you back today or tomorrow, you gotta keep trying.”

The Respondent acknowledge that there were days after he was declared fit for restricted duty that he went to work not feeling very well. He further acknowledged that after he was declared fit for restricted duty he went to work and did not complete his tour. When asked how long he did work before he went sick on those days, he stated that,” it would vary two to six hours with - - depending when my pain occurred.” He added that at that time his tours were about eight hours, consisting of 0700 by 1523 hours. He acknowledged that there were times that he would make it until about 1300 before he went sick. When asked how he felt at those times before going sick, he stated that, “I couldn’t concentrate, I was very extremely agitated, there was - - I just couldn’t function. I was having a hard time just doing the most simplest tasks. My handwriting is horrible now because of my arm so I mean there are just so many different things going on with me.” He stated that the pain affects the use of his arms and legs even at desk work “completely.”

When asked to describe how the pain takes over or affects him he stated when he appears “at work I am already in pain, increased pain because of the drive to the 107

which is on the border on Nassau from Staten Island and my back would already be in full burn. A lot of pain, full pain increased in my back. I would go in I wasn't allowed to be in civilian attire so I would have to actually change and then I would come up stand roll call or be told to go to the desk and do desk duties and there would be just one step after the next. It would just slowly start increasing to the point where I wasn't able to do even the simplest task. I was having a hard time filing reports, answering the phone, understanding people over the phone because of the headaches. I was also getting headaches also, eye aches then it was just a clear, purely a struggle it wasn't - - it was really hard for me to do."

The Respondent testified that at one point he decided to put in for a disability pension or three quarters pension for the on-the-job injury. When asked to explain why he decided to do that, he told the Court that, "I believe maybe five months post-op and I had exhausted every possible remedy to improve my back. Lieutenant Valenti was pretty much riding me hard every time I went sick and he threatened to survey me off as well and being that I was injured on the job I felt that his threats were real and I went to speak to the Article II counselors and they told me to put it in. He further stated that Valenti "would come up and tell me that he is getting calls from my command that he is getting tired of me going sick and that if I continue going sick he is going to suspend me." He said that this conversation took place in the middle of August 2008, and he remembered that one conversation took place August 11, 2008.

According to the Respondent's testimony, one of the main things that caused the Respondent to put in his application for accident or disability retirement was that Valenti told him that he was going to 'survey him off the job.' He stated that because of the

nature of his injury he took a lot of offense to Valenti telling him that he was going to do this, because he felt that it was a punishment. He explained that, "I am trying to fight back to full duty to be 100 percent. I would give anything to be 100 percent and be a police officer again and was just an insult and a slap to my face and when he told me that he was going to retire me off at 50 percent disability. I don't think I deserve that." The Respondent acknowledged that that is what Valenti meant when he said that he was going to retire him at 50 percent. He acknowledged that being surveyed off the job at 50 percent would be in reference to an off-duty occurrence that caused the disability. The Respondent further stated that he feels that the problems with his neck and his back started with the incident back in January of 2001.

The Respondent acknowledged that on August 14, 2008 he had a locker at the 107 Precinct. He further acknowledged that he had a lock attached to that locker and on the back of the lock there was a serial number that he was aware of. When he was asked if he filed off the serial number so that it was completely unable to be read on the lock that he had back in August 2008, he replied "no, I did not." When asked if he was aware that there was a Patrol Guide provision which requires him to file off the serial number of the lock he replied "I am aware of it now. I am definitely aware of it now." He further explained that "that was during the time when I was in a lot of pain traveling to work unable to take medicine, unable to concentrate and I had forgotten my combination to my lock and I had put a new lock on there and I completely forgot to scratch it off."

On cross-examination the Respondent testified that he lives with his wife and two children. He states that he has twin boys that are five-years old. He further stated that his wife is the primary care giver for his sons and that "she works outside the home maybe

one to five hours a day depends on the weather or not if my day off or the boys are in school.” He stated that his sons had been diagnosed with autism and his son Zack has a medium case of the condition and that his son Mathew is improving from a medium case of the same condition. He acknowledged that having a medium case of the condition means that sometimes they act out or act up a little bit.

He reiterated that his wife works outside the home and that “she works maybe an hour during the times that they were at school and then on my RDO depending on how I feel she would ask me if she could stay longer.” He acknowledged that if his wife has to work no one else watches his children but him. When his wife leaves the house on weekends he watches the children. The Respondent acknowledged that between the dates of March 1998 and November of 2000, he had another line of duty injury with his knee and had surgery on his knee that caused him to be out Chronic A sick which means he was out at least eight times during that time period. He stated that he had surgery on his knee and that while he had a lot of complications with the knee, it is much better now.

The Respondent testified that he goes to physical therapy in Staten Island approximately 10 to 15 minutes from where he lives. He states that he attends physical therapy on Mondays, Wednesdays and Fridays and that while Wednesday is his day off, on Monday and Friday, while he reports to work, he was permitted to leave two hours early during the period from July 2008 to September 2008 to attend the physical therapy. He explained that “I was authorized to leave work depending on the therapist I was seeing the only times that were mostly open with her were about 2:30ish so I would leave work about 1:30 so it is about two hours.” He stated that he would not go back to work after therapy. He acknowledged that on those Mondays and Fridays when he went to

therapy, the last two hours of his tour were actually spent traveling to physical therapy and going to physical therapy.

The Respondent stated that he had to drive to work because he could not take public transportation. He stated that the public transportation, that being taking the ferry and taking the train and then the bus to the 107 Precinct "would be horrendous on my back."

With regard to the Respondent's military duty he stated that he spent four years in the Navy and then after active duty he spent additional time for a total between 10 to 11 years totaled in the reserves. He stated that he re-enlisted in the Navy reserve "to try to complete my pension with the military." He added that he spent approximately 10 years total amount of time in the military. He believes after his discharge he finished his association with the Navy in approximately 2003 or 2004. When asked how he could remain in the Navy reserves after his injury in January 2001 until 2003 or 2004 given the type of pain that he claims that he was in from the injury and how could he stay in the reserves for three or four more years he stated "I was always in pain when I was in the reserves and I actually did get re-injured when I was in the reserves also. I re-injured my back in the reserves yes." When he was again asked how he could remain in the Navy reserves after he had that horrible injury to his neck and back while on duty with the Department he replied "like I said Sir, being a Police Officer and being in the military are two things that I loved in my life so I mean I was trying to make the sacrifice."

He stated when he went to Lefrak City to see the Medical District doctors "it was always Dr. Galvin unless he was out." With regard to the Department orthopedics it was Dr. Miller that he would see. He stated that he saw Dr. Goldman in 2008 several times

but "I am strictly forbidden [by the Medical Division] from seeing any other doctor besides Dr. Miller unless they put me in an office with another doctor." He explained "myself and Dr. Miller had a falling out whether or not I had the - - I should have the surgery and I tried to go above him to see the chief surgeon and was told that I had to see Dr. Miller."

On questioning from the Court concerning who was his private physician the Respondent stated Dr. Shau. The Respondent acknowledged that Dr. Shau was not connected with the Department and that he was not ordered not to see his private doctor. He further acknowledged that he could have went back to see Dr. Shau any time he wanted.

The Respondent stated that when he went to Lefrak City he did not drive all the time. He stated "I did have my next door neighbors take me several times, my brother took me several times, my wife took me several times but she has her plate full so I would not ask her unless I absolutely needed her."

With regard to his Naval Reserve duty, the time that he had to commit to fulfill his obligation was "two days of the month and it was all classroom." When asked by the Court if he had reserves duty two weeks out of each summer he replied "Yes, Sir and that was a lot of training in the classroom." He acknowledged that by classroom training it meant sitting at a desk in a classroom.

FINDINGS AND ANALYSISDisciplinary Case No. 84579/08Specifications 1, 2, 3, 4, 5 and 6

It is charged that the Respondent while assigned to the 107 Precinct on about eight (8) occasions between about July 28, 2008 and about September 5, 2008, while designated as "Chronic Sick – Category B," and while assigned on restricted duty status he failed and neglected to perform his assigned tours as directed by and after having been found fit by Department Medical Division physicians in that on each occasion, he reported sick during his assigned tours or within a few hours of the commencement of his assigned tours, claiming sickness due to the existing condition that was reviewed and evaluated the previous day by Medical Division physicians.

It is further charged that the Respondent on July 28, 2008, having been found fit for restricted duty and ordered by Department Orthopedist Arnold Goldman, M.D., to report to work to perform his next scheduled tour of duty, he wrongfully failed to comply with said order.

He is also charged that on August 6, 2008, August 15, 2008, August 29, 2008 and September 2, 2008, having been found fit for restricted duty and ordered by Department Special Medical District Surgeon Peter Galvin, M.D., to report to work to perform his next scheduled tour of duty, he wrongfully failed to comply with said order.

In essence the Respondent is charged with not adhering or obeying the directives of Department Surgeons to perform restrictive duty after being found fit to do so.

At the outset, and contrary to the arguments made by Respondent's counsel, directives by Department Surgeons to members of the Department to report for restricted

duty are lawful orders. Patrol Guide section 203-01 lists the “order of rank in the police service.” One of the ranks listed is “Inspector” and further states that “Police Department ...Surgeons have the assimilated rank of Inspector.” Patrol Guide section 203-03, under “Compliance with Orders” states “Obey lawful orders and instructions of supervising officers.” Doctors Goldman and Galvin both have the assimilated ranks of Inspectors and, as such, are supervising officers as are Miller and Lichtenstein. Therefore, in the opinion of this Court their instructions, directions, recommendations and even their suggestions to members to perform certain duties are orders by them to perform that duty. They do not need to say “I order you to perform...,” nor do they, as doctors, need to always be mindful that their instructions to members are orders. It is the fact that they hold the rank of Inspector that informs members to obey their instructions in a military fashion—as orders.

Moreover, Patrol Guide section 205-45, states in relevant part that “Category B members fit for duty after examination by the Special Medical District surgeon and who report sick again for the same reason prior to the next scheduled tour duty may be suspended without pay. If upon subsequent examination by the surgeon no objective findings such illness or injury are found charges and specifications will be prepared by the commanding officer of the Medical Division.” While the Respondent is not charged with this section, it does inform this Court that an examination and findings of fitness by the District Surgeon is binding without an objective finding of illness or injury that is contrary to the Surgeon’s decision.

The Respondent suffered a line-of-duty injury in 2001 when he had an altercation with a prisoner. From that incident he suffered injuries to his back and neck and

subsequently underwent treatment to relieve pain and aid in recovery. He re-injured himself through subsequent physical activity and in April 2008 he underwent surgery in an attempt to correct the problem. (RX A) The Respondent, however, still experienced pain and discomfort from the afflicted area.

The Respondent has been designated as Chronic B sick since 2001, with the exception of a six month period. Chronic B designation is when a member reports sick six times in a twelve month period or when a member is out sick a total of more than forty days in a twelve month period regardless of the number of events.

Goldman and Galvin testified as to their evaluations of the Respondent's condition and their determinations and "instructions" for him to report for restricted duty.

Goldman is a practicing orthopedic surgeon since 1981 and is familiar with the type of injury and surgery that was performed on the Respondent. Goldman first saw the Respondent on July 8, 2008 at the request of Galvin to determine whether he could go back to restricted duty. He reviewed the Respondent's medical file along with asking the Respondent if he had any notes from his doctors, therapists, any MRIs or CAT scan results. Goldman had no independent recollection as to whether such notes were provided by the Respondent.

Goldman's evaluation of the Respondent was that he had disc replacement surgery, that he was able to sit in a car to get to the examination, that portions of his spine and neck motion were limited, but that he was able to bend his neck up and down and from side to side, that he goes to therapy and that he did not "limp to and from the office." He determined that the Respondent had overall good range of motion on both upper extremities. He did not document anything regarding the Respondent's pain. He

explained, however, the fact that an individual is in pain is not prohibitive of that individual being returned to restrictive duty, since it, "is not uncommon for somebody to have pain several months after surgery. Even from various sprains, aches, anyone could have pain and be able to do sitting work similar that he is doing at home." He determined that it was a "good sign" that he was able to sit in the car and able to go to therapy two or three times a week, and that he should be able to go to work and do sitting type of work. Goldman further recommended more physical therapy and that it was his opinion that the Respondent should be able to return to restricted duty at the end of July 2008. Goldman said that he considered that length of time (three months post operation) appropriate based on his experience with over one thousand cases involving neck and back surgeries.

Goldman also saw the Respondent on July 28, 2008, and he did not record that the Respondent told him anything new or unusual on this date. He recommended restricted duty for the Respondent on this date "Based on the fact that he was still able to go to physical therapy, sit in the car to get to Lefrak. I was aware that he was on the same medication, if I would have seen anything significant I would have documented that, but it was a similar examination that I had seen before." He returned the Respondent to work as of August 3, 2008. Goldman made a point of saying that if the officer says something of significance in response to his determination he would document those statements. Goldman stated that the Respondent made no such statements nor did he ask for a further examination.

Goldman also saw the Respondent and filled out a sick report for the Respondent on August 18, 2008. Goldman read the following notes from the report: 'Well healed wound noted over base of right side of his neck today, clean and dry actually lower back

feels worse than neck, overall externally rotate behind head, rotate bilaterally, able to go to physical therapy, either gets a ride from a neighbor or father-in-law drives him, okay for pain management as well as follow up X-rays.” When asked, Goldman told the Court that he had probably determined that the lower back felt worse by asking the Respondent. Goldman testified that as a result of his examination he had determined that the Respondent should be able to go back to restricted duty as of August 18, 2008 at 1523 hours, essentially the next tour of duty after the day that he saw him.

It should also be noted that Valenti testified that the Respondent was evaluated on two occasions by the Department’s Dr. Lichtenstein and was directed to perform restricted duty on August 11, 2008 and August 25, 2008.

Galvin met with and evaluated the Respondent’s condition on five separate occasions: August 6, 2008, August 15, 2008, August 29, 2008, September 2, 2008 and September 5, 2008. On each of those occasions he observed the Respondent, asked him questions about his condition, made notes of the Respondent’s complaints and reviewed his file which consisted of his physical examinations, letters from treating physicians, the Article II minutes and the opinions of other Department specialists including the orthopedic surgeons. During each meeting Galvin stated that the Respondent’s condition did not change from the last meeting. While the Respondent complained of pain to the affected area, Galvin concluded that the pain was consistent with his injury. Galvin also noted that the Respondent was attending physical therapy and was able to drive to Lefrak City. At the end of each of those meetings Galvin concluded that the Respondent was fit to perform restricted duty.

Galvin explained that there are three types of duty that he can assign an officer: full duty, limited duty, and restricted duty. Limited duty is usually for a specific amount of time, up to six weeks, and then there is restricted duty, which is the same as limited duty only intended for a longer term. The type of duty that someone on limited or restricted duty would participate in would be determined by the person's command, but in general the person is not permitted to do patrol or to drive a Department vehicle, and is instead assigned to some sort of administrative duty within the confines of the command. Galvin noted that even if someone is deemed to be disabled by the NYPD Article II Medical Board and eligible for a disability retirement, they are usually still capable of working in a restricted capacity, and that even though the Respondent in this case has been approved for a disability retirement, that determination would not have anything to do with whether or not the Respondent can perform limited or restricted duty. Galvin explained that the duty the disability retirement speaks to is only full duty, and not restricted or limited duty.

Galvin's conclusions took into account the fact that while the Respondent complained that he was in pain when he sat in a chair, he was in fact able to sit in traffic and drive for approximately an hour. He also stated that a member on restricted duty is not being directed to stay strapped in a chair for an entire tour of duty and could be doing any number of things within the confines of the command.

At one point, Galvin had the Respondent see Dr. Miller a Department orthopedic surgeon. He did so because "if I'm seeing the person two or three times a week for the same problem and he is telling me nothing has changed, then I think it would be reasonable to assume that the examination wouldn't change," and he therefore believed

that since he was not an orthopedic specialist that it was a reasonable approach to seek out another orthopedic surgeon's opinion, to ensure that he did not miss something. Miller's opinion, like Galvin's, was that the Respondent was capable of performing restricted duty.

The Respondent, however, claimed that the Medical District doctors' examinations were superficial. He said that he walked into Goldman's office and was told to do "a couple of raises with my arms at most and asked me to walk back to the door and that was it." He was then asked to wait outside. He further stated that when he went to see Galvin he would walk into his office and "I would tell him what occurred that day whether it was during the drive in to work, at work, what occurred with my pain and I would stand there. He would offer me to sit I would tell him I just sat in the car this whole time there is no way I can sit down and he would tell me to keep trying; "I'm putting you back today or tomorrow you gotta keep trying."

The Respondent described the work he did on restricted duty as "they were asking me to answer phones, act as an assistant desk officer, T/S operator, a lot of reaching involved, a lot of lifting up the phone. Anything to do with my arms was painful they were asking me to fingerprint and there were days that I just couldn't do it and I would tell them that they have to find someone else because of leaning and everything."

This Court, after a reviewing all of the evidence, finds that a significant weight should be given to the testimony of the Department doctors in their assessment of the Respondent's condition. With their considerable combined experience they evaluated and determined that the Respondent's condition, including the pain he was experiencing and the work he was expected to perform, was able to do the minimal amount of work

that is required of a member on restricted duty. While the Respondent claims that he could not perform that duty on the days in question, he was nevertheless able to drive for extended periods of time, report to physical therapy three times a week and on his days off take care of and meet the needs of his two young sons when his wife was out of the home. This Court therefore finds that while he was readily able to perform functions not associated with his work, he disobeyed the directions of Department doctors to perform the very limited tasks presented to him on restricted duty.

Accordingly, I find the Respondent Guilty as charged.

Disciplinary Case No. 84986/09

The Respondent is charged that while assigned to the 107 Precinct, on or about August 14, 2008, he wrongfully secured his Department locker with combination lock displaying the serial number of said lock.

The Respondent admitted during his testimony that he in fact secured his locker with a combination lock that displayed the lock's serial number in violation of the Patrol Guide.

Accordingly, I find the Respondent Guilty as charged.

PENALTY

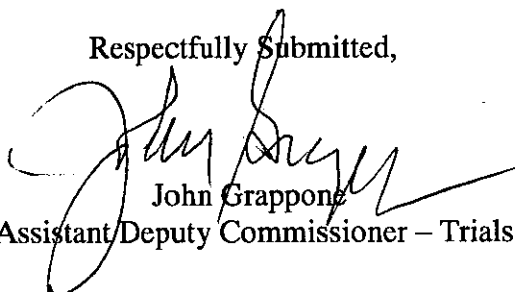
In order to determine an appropriate penalty, the Respondent's service record was examined. See Matter of Pell v. Board of Education, 34 N.Y.2d 222 (1974). The Respondent was appointed to the Department on July 18, 1996.

The Respondent has been found Guilty under six charges and specifications of failing to perform his assigned tours after being found fit to perform restricted duty by Medical Division physicians. He also has been found Guilty of securing his locker with a combination lock that displayed the lock's serial number.

The Department has recommended that the Respondent be terminated from his position as a police officer. This Court, however, finds that termination is too harsh a punishment under the circumstances of this case. The Respondent has an injury to his back and neck that causes him pain. While he disobeyed the directives of Department doctors to perform restricted duty, he was legitimately diagnosed for that limited duty. As such a penalty that is less than termination is appropriate.

Accordingly, based on the foregoing and the Respondent's service record, I recommend that the Respondent be DISMISSED from the New York City Police Department, but that his dismissal be held in abeyance for a period of one year, pursuant to Section 14-115 (d) of the Administrative Code, during which time he remains on the force at the Police Commissioner's discretion and may be terminated at anytime without further proceedings. I further recommend that the Respondent forfeit 60 vacation days.

Respectfully Submitted,


John Grapponi
Assistant Deputy Commissioner – Trials

